

Safeguarding Adults Policy

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Responsible Director:	Chief Nursing Officer
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1. Introduction

Safeguarding adults requires effective working in a multi-agency partnership and is an integral part of the care the Trust provides. Staff are working in conjunction with the following frameworks within the West Midlands Adult Safeguarding Policy and Procedure requirements within Coventry, Warwickshire, and Solihull.

The Care Act (2014) colloquially known as “the Act” identifies adult safeguarding as ‘protecting a person’s right to live in safety, free from abuse and neglect’.

This policy sets out the ways in which the Trust supports and promotes the safeguarding/protection of adults in practice and identifies the Trust responsibilities for all its employees including the processes they need to consider and adhere to at a local, regional, and national level.

The Trust is compliant with the Act by their commitment to cooperate with all relevant local authorities to safeguard adults. All staff working for the Trust has a responsibility to identify, assess and respond to safeguarding concerns. These duties apply in relation to any person who is aged 18 or over. This includes someone who is over the age of 18 who is in receipt children’s services where a safeguarding concern is raised.

The Care Act (2014) identifies that the local authority is required to make enquires if it believes an adult is at risk of abuse or neglect. All staff members have a duty to report safeguarding concerns and gain support and advice from the Trust Safeguarding Team. The Trust works in partnership with both Coventry and Warwickshire local authorities under Section 75 arrangements of the NHS Act 2006. It is the formal partnership agreement between the Trust, Coventry City Council and Warwickshire County Council. The formal agreement pertaining to the safeguarding of adults within the Trusts Mental Health Services is within schedule 15 of this agreement.

2. Purpose

The purpose of this policy is to ensure that when adult abuse is disclosed, suspected or apparent, Trust staff members are able to respond effectively and appropriately.

2.1 Principles

The following six principles set out in the Care Act 2014 should underpin all safeguarding adult work for the Trust.

- **Empowerment:** People being supported and encouraged to make their own decisions and informed consent. “I am asked what I want as the outcomes from the safeguarding process, and these directly inform what happens.”
- **Prevention:** It is better to act before harm occurs. “I receive clear and simple information about what abuse is, how to recognise the signs and what I can do to seek help.”

- **Proportionality:** The least intrusive response appropriate to the risk presented. “I am sure that the professionals will work in my interest, as I see them, and they will only get involved as much as needed.”
- **Protection:** Support and representation for those in greatest need. “I get help and support to report abuse and neglect. I get help so that I can take part in the safeguarding process to the extent to which I want.”
- **Partnership:** Local solutions through services working with their communities. Communities have a part to play in preventing, detecting, and reporting neglect and abuse. “I know that staff treat any personal and sensitive information in confidence, only sharing what is helpful and necessary. I am confident that professionals will work together and with me to get the best result for me.”
- **Accountability:** Accountability and transparency in delivering safeguarding. “I understand the role of everyone involved in my life and so do they.”

3. Definitions

3.1 Adult

An adult is any person who has attained their eighteenth birthday and thereafter.

3.2 Abuse

The Care Act (2014) identifies abuse in the different categories below and acknowledges patterns of abuse can be **serial, long term or opportunistic abuse**. If the abuse is caused by a child/ren the local authority children services will also need to be involved.

3.2.1 Physical Abuse

When an adult is subject to assault, hitting, slapping, pushing, scalding, or burning, misuse of medication, restraint, or inappropriate physical sanctions.

3.2.2 Sexual Abuse

Can include rape, indecent exposure, sexual harassment, inappropriate looking or touching, sexual teasing or innuendo, sexual photography, subjection to pornography or witnessing sexual acts, indecent exposure and sexual assault or sexual acts to which the adult has not consented or was pressured into consenting.

3.2.3 Psychological Abuse

Can include emotional abuse, threats of harm or abandonment, deprivation of contact, humiliation, blaming, controlling, intimidation, coercion, harassment, verbal abuse, cyber bullying, isolation or unreasonable and unjustified withdrawal of services or supportive networks.

3.2.4 Financial or Material Abuse

Also referred to as economic abuse, this can include; theft, fraud, internet

scamming, coercion in relation to an adult's financial affairs or arrangements, including in connection with wills, property, inheritance or financial transactions, or the misuse or misappropriation of property, possessions, or benefits.

3.2.5 Modern Slavery

Encompasses slavery, human trafficking, forced labour and domestic servitude, female genital mutilation. Traffickers and slave masters use whatever means they have at their disposal to coerce, deceive, and force individuals into a life of abuse, servitude, and inhumane treatment.

3.2.6 Discriminatory Abuse

Including forms of harassment, slurs, or similar treatment; because of race, gender and gender identity, age, disability, sexual orientation, or religion.

3.2.7 Organisational Abuse

Including neglect and poor care practice within an institution or specific care setting such as a hospital or care home, for example, or in relation to care provided in one's own home. This may range from one off incidents to on-going ill-treatment. It can be through neglect or poor professional practice because of the structure, policies, processes, and practices within an organisation.

3.2.8 Neglect and Acts of Omission

Including ignoring medical, emotional, or physical care needs, failure to provide access to appropriate health, care and support or educational services, the withholding of the necessities of life, such as medication, adequate nutrition, and heating.

3.2.9 Self-Neglect

This covers a wide range of behaviour neglecting to care for one's personal hygiene, health, or surroundings and includes behaviour such as hoarding.

3.2.10 Domestic Abuse

The statutory definition of domestic abuse covers any incident or pattern of violence or abuse between those aged 16 and over who are personally connected to each other. Abusive behaviour can include physical or sexual abuse, violent or threatening behaviour, controlling or coercive behaviour, economic abuse and psychological, emotional, or other abuse. Further information on domestic abuse and the offence of controlling or coercive behaviour can be found in the Controlling or Coercive Behaviour: Statutory Guidance Framework (Home Office 05th April 2023 and in the Domestic Abuse Act (2021) Statutory Guidance (Home Office July 2022 updated April 2023). Domestic abuse also includes so called 'Honour' based abuse.

3.2.11 Exploitation

Criminal exploitation of children and vulnerable adults is a widespread form of harm and is a typical feature of "county lines" activity (Home Office 2018).

3.3 Wider Safeguarding Implications

The Care Act (2014) also recognises the wider safeguarding implications:

3.3.1 Forced Marriages

A forced marriage is a marriage in which one or both spouses do not consent to the marriage but are coerced into it. Force can include physical, psychological, financial, sexual, and emotional pressure. In the cases of vulnerable adults who lack the capacity to consent to marriage, coercion is not required for a marriage to be forced (multi-agency statutory guidance for dealing with forced marriage and multi-agency practice guidelines: Handling cases of forced marriage {accessible version} Updated 13 April 2023)

An arranged marriage is not the same as a forced marriage. In an arranged marriage, the families take a leading role in choosing the marriage partner. The marriage is entered into freely by both people.

3.3.2 Hate Crime

The Home Office's definition of a hate crime is:

"Any incident, which constitutes a criminal offence, which is perceived by the victim or any other person as being motivated by prejudice or hate".

Hate crime is any criminal offence committed against a person or property that is motivated by an offender's hatred of someone because of their:

- Race, colour, ethnic origin, nationality, or national origins
- Religion
- Gender or gender identity
- Sexual orientation
- Disability.

Hate crime can take many forms including:

- Physical attacks: such as physical assault, damage to property, offensive graffiti, neighbour disputes, and arson.
- Threat of attack: including offensive letters, abusive or obscene telephone calls, groups hanging around to intimidate and unfounded, malicious complaints.
- Verbal abuse or insults: offensive leaflets and posters, abusive gestures, dumping of rubbish outside homes or through letterboxes, and bullying at school or in the workplace.

3.3.3 Pressure Ulcers

A pressure ulcer is defined as localised damage to the skin and or underlying tissues because of pressure or pressure in combination with shear. Pressure ulcers occur over bony prominences but may also be related to a medical device or other object (Guidance: Pressure ulcers: applying All Our Health Information for healthcare professionals to protect the public from pressure ulcers {bed sores} (Gov.UK 01st April 2015 updated 09th June 2022)

3.4 Section 75 Arrangements

This refers to Section 75 of the NHS Act 2006 and is the formal partnership agreement between the Trust, Coventry City Council and Warwickshire County Council. This sets out the Trusts responsibilities for safeguarding adults with acute mental health needs or older persons with mental health needs within CWPT services.

3.5 Capacity

The ability of an individual to understand, retain and weigh in the balance information relating to a specific decision at a specific time.

3.6 Deprivation of Liberty Safeguards (DoLS)

The safeguards in place within the Mental Capacity Act 2005 which seek to ensure that any restriction on the movement or activity of those people who lack capacity is both proportionate to potential risks and in the best interest of that individual.

3.7 Significant Harm

'Ill treatment (including sexual abuse and forms of ill treatment that are not physical), but also the impairment of, or an avoidable deterioration in physical or mental health: and the impairment of physical, intellectual, emotional, social, or behavioural development.' Law Commission (1995).

3.8 Human Rights

All persons have the right to live their lives free from violence and abuse. This right is underpinned by the duty on public agencies under the Human Rights Act (1998) to intervene proportionately to protect the rights of citizens. These rights include:

- Article 2: "The Right to Life"
- Article 3: "Freedom from Torture" (including humiliating and degrading treatment); and
- Article 8: "Right to Family Life" (one that sustains the individual).

3.9 Serious Incidents

In broad terms, serious incidents are events in health care where the potential for learning is so great, or the consequences to patients, families and carers, staff or organisations are so significant, that they warrant using additional resources to mount a comprehensive response. Serious incidents can extend beyond incidents which affect patients directly and include incidents which may indirectly impact patient safety or an organisation's ability to deliver ongoing healthcare.

All serious incidents must be processed according to Trust Incident Reporting Policy.

3.10 Safeguarding Adult Reviews (SARs)

Safeguarding Adults Reviews (SARs) are carried out to establish whether there are lessons to be learnt, identify clearly what those lessons are, and what is expected to change as a result; to improve inter-agency working and better safeguard and promote the welfare of vulnerable adults. SARs are not inquiries into how a vulnerable adult died or who is culpable. If a SAR is thought to be appropriate reference should be made to localities interagency policy and procedures.

3.11 Offensive Weapons Homicide Reviews (OWHRs)

The Police, Crime, Sentencing and Courts Act 2022 introduced a requirement on the police, local authorities, and Integrated Care Boards in England to review the circumstances of certain homicides where the victim was aged 18 or over, and the events surrounding their death involved, or were likely to have involved, the use of an offensive weapon. The intention is that these new reviews will improve the national and local understanding of what causes homicide and serious violence, better equipping services to prevent weapons-enabled homicides and, in so doing, save lives.

3.12 Freedom to Speak Up (Colloquially known as “Raising Concerns / Whistle Blowing”)

This refers to workers being able to speak up about anything that is getting in the way of delivering good, safe patient care or positive staff experience. This could include anything that doesn't feel right, such as a way of working or process that puts patients at risk, or inappropriate, unprofessional behaviour or actions that affect patient care or the safety and wellbeing of workers. It could also be sharing an idea for improvement or speaking up at the time to prevent potential harm. Our Freedom to Speak Up: Speak up, listen up, follow up Policy provides a framework to help normalise speaking up for the benefit of patients and workers and ensure all matters raised are captured and considered appropriately whether raised informally or formally

3.13 Procedure for Sharing Information with the Coroner

Information for His Majesty's Coroner is submitted by the Patient Safety team and details kept on file. Any information to be shared, following approval/agreement, should be submitted to the Patient Safety team with details as to which Coroner's office has requested the information.

3.14 Safeguarding Enquiries

An enquiry is the action taken or instigated by the local authority in response to a concern that abuse, or neglect may be taking place. An enquiry could range from a conversation with the adult, or if they lack capacity, or have substantial difficulty in understanding the enquiry their representative or advocate, prior to initiating a formal enquiry under Section 42, right through to a much more formal multi-agency plan or course of action.

3.15 Think Family

The Think Family agenda recognises and promotes the importance of a whole-family approach. The principles include:

- No wrong door – contact with any service offers an open door into a system of joined-up support. This is based on more coordination between adult and children's services.
- Looking at the whole family – services working with both adults and children consider family circumstances and responsibilities. For example, an alcohol treatment service combines treatment with parenting classes while supervised childcare is provided for the children.
- Providing support tailored to need – working with families to agree a package of support best suited to their particular situation.

- Building on family strengths – practitioners work in partnerships with families recognising and promoting resilience and helping them to build their capabilities (Social Care Institute for Excellence {SCIE} 2011).

3.16 Section 42

Enquiry by local authority:

- (1) This section applies where a local authority has reasonable cause to suspect that an adult in its area (whether ordinarily resident there):
 - (a) Has needs for care and support (whether the authority is meeting any of those needs)
 - (b) Is experiencing, or is at risk of, abuse or neglect, and
 - (c) As a result of those needs is unable to protect himself or herself against the abuse or neglect or the risk of it.
- (2) The local authority must make (or cause to be made) whatever enquiries it thinks necessary to enable it to decide whether any action should be taken in the adult's case (whether under this Part or otherwise) and, if so, what and by whom.
- (3) "Abuse" includes financial abuse; and for that purpose, "financial abuse" includes:
 - (a) Having money or other property stolen,
 - (b) Being defrauded,
 - (c) Being put under pressure in relation to money or other property, and
 - (d) Having money or other property misused.

3.17 Decision Making Managers

The manager is the professional responsible for making the decision as to whether the safeguarding concern proceeds to enquiry and for overseeing the safeguarding process. This would usually be a registered professional who has managerial responsibilities.

3.18 Independent Advocate

An independent advocate is an individual who supports the adult during a safeguarding enquiry/activity. Their role is supporting and enabling the person to:

- Express their views and concerns
- Access information and services
- Defend and promote their rights and responsibilities
- Explore choices and options.

4. Duties / Responsibilities

4.1 Chief Executive Officer

The Chief Executive Officer has overall responsibility for the Trust's safeguarding of adults. The operational responsibility for the Safeguarding Adults Policy is delegated to the Chief Nursing Officer.

4.2 Chief Nursing Officer

The Chief Nursing Officer is responsible to the Trust Board and the Chief Executive Officer in relation to the Safeguarding Adults Policy of Coventry and Warwickshire Partnership NHS Trust, they are the Accountable Officer and provide regular reports to the Trust Board in this regard.

4.3 Caldicott Guardian

The Caldicott principles and recommendations apply specifically to patient identifiable information and emphasise the need for controls over the availability of such information and access to it. In particular, a Caldicott Guardian, appointed in each NHS organisation, has specific responsibilities to oversee an ongoing process of audit, improvement, and control.

4.4 Head of Safeguarding (Children and Adults)

The Head of Safeguarding (Children and Adults) supports the Chief Nursing Officer in the discharging of safeguarding functions and has a role in ensuring that the Trust meets its statutory organisational responsibilities in relation to Safeguarding Children and Adults with care and support needs and provides management oversight of the functions of the Safeguarding Team. The Head of Safeguarding is the “Prevent” Lead and provides day to day advice and support where there are concerns about children or adults being groomed into involvement with violent extremism or terrorism related concerns.

4.5 The Safeguarding Team

The Safeguarding Team are responsible for advising all staff throughout the organisation on issues relating to safeguarding activities, which will include:

- Specific clinical advice
- Support and supervision in respect of complex cases
- Provision of training
- Production of policy
- Audit
- Serious case/ learning reviews
- Data collection
- Ensuring concordance with Section 75 arrangements.

4.6 Operational/Service/Team Managers will:

- Provide support for staff, patients and carers as required.
- Ensure that all new employees attend Trust induction for Safeguarding Adults and Health Workshop to raise Awareness of PREVENT as part of Trust Onboarding on recruitment to the post.
- Ensure that staff have access to Trust policies and interagency policies relating to the safeguarding of adults e.g., paper format, Intranet, Internet accessibility.
- Ensure that staff have access to the appropriate forms relating the safeguarding of adults.

- Ensure that all members of the team are appropriately trained and to monitor and record attendance of their staff group and ensure that they attend adult protection awareness training as part of Onboarding on recruitment to the post of a member of staff joining the Trust.
- Ensure that their staff have access to the appropriate supervision relating to adult safeguarding.
- In the case of safeguarding enquiries, whatever the course of subsequent action, the professional concerned should record the concern, the adult's views and wishes, any immediate action has taken and the reasons for those actions. The purpose of the enquiry is to decide whether or not the local authority or another organisation, or person, should do something to help and protect the adult.

4.7 All Staff

All staff must comply with the Trust's Safeguarding Policy and Procedures. Staff must be aware of their duty to recognise and act on concerns about safeguarding children and adults irrespective of whether they are primarily working with adults or with children / young people and to "Think Family (Department for Children, Schools, and Families {DCSF} (2008)

In addition, staff must work within the Adult Safeguarding: Multi-agency policy & procedures for the protection of adults with care & support needs in the West Midlands requirements within Coventry, Warwickshire, and Solihull.

Staff must ensure that they access and receive appropriate safeguarding supervision relating to safeguarding concerns and also following any serious incidents.

Where staff members engage in personal relationships with a service user (past or present) the staff member must inform their manager immediately who will ensure appropriate arrangements can be put in place. For further policy requirements please refer to Appendix 1.

4.8 Safeguarding Group

The Trust's Safeguarding Group (Operational/Strategic) will meet on a quarterly basis to review the safeguarding of adults and their families who are in our services.

4.9 Local Safeguarding Adults Partnership / Board (SAP/B)

The role of the Local Safeguarding Adult Partnership / Board is established in the Care Act 2014. The responsibility and function of the local Safeguarding Adults Partnership / Board is to ensure that agencies and partners work together to safeguarding adults at risk of harm or abuse and that safe arrangements are in place. It must:

- Publish a strategic plan for each financial year setting out its main objectives and how they will be achieved.
- Publish an annual report identifying what it has done to achieve the objectives.
- It must conduct Safeguarding Adult Reviews.

4.10 Independent Advocate

The role of the Independent Advocate is to support and enable the person to:

- Express their views and concerns
- Access information and services
- Defend and promote their rights and responsibilities
- Explore choices and options.

5. Process

5.1 Immediately on becoming aware of a possible safeguarding adult concern, the individual member of staff will ensure the safeguarding process is in accordance with the Local Safeguarding Adult Partnership / Boards and West Midlands Adults Safeguarding Policy and Procedures in the location of where the abuse occurred. i.e., all Trust staff must work within the West Midlands Adult Safeguarding Policy and Procedures within the Coventry, Warwickshire, and Solihull area. Please see Appendix 2.

5.1.1 Adult Mental Health Services

Adult Mental Health Services undertake the responsibility under Section 75 arrangements to make enquiries to safeguard adults and fulfil the statutory requirements of the Care Act 2014. These responsibilities include:

- Ensuring the procedures laid out in the West Midlands Adults Safeguarding Policy and Procedure are implemented and followed.
- Ensuring effective multi-agency working and information sharing is carried out, including Section 42 of the Care Act 2014 requirements.
- Ensuring and checking that Partner Agencies follow through agreed actions.
- Keeping full records of the Safeguarding Adults Process and outcomes in line with their internal agency record-keeping processes.
- Reporting aggregate data.
- Reporting aggregate data on the incidence and profile of Safeguarding Adults activity to the local Safeguarding Adults Partnership / Board in line with local and national reporting requirements.
- Undertaking appropriate case file audit and quality assurance exercises, and reporting outcomes and recommendations to the local Safeguarding Adults Partnership /Board or nominated subgroup.

Please see Appendix 3 for further information on:

1. Guidance
2. Chronology
3. S42 Enquiry Report
4. Meeting Minutes Template
5. Recording and completing of Safeguarding Concerns and outcomes of Section 42 Enquiries within Carenotes

5.1.2 All Other Services

The staff member should adhere to the Care Act 2014 “Making Safeguarding Personal” and follow procedures as identified within the West Midlands Adult Safeguarding Policy and Procedures to ensure they discuss any safeguarding concerns with the adult and family/carers or advocate and where possible seek agreement. **This should only be done where such discussions will not place the adult at increased risk of significant harm or cause any significant delay.**

All safeguarding adult concerns reported to the local authority **must** be copied to the Trust’s Safeguarding Team.

The individual staff member who has a concern has responsibility for ensuring they receive feedback and are aware of the outcome of that concern. **This includes ensuring contact with the local authority if no response is received.**

It is essential that all serious incidents and near misses, relating to the abuse of an adult are reported as incidents as per the Trust’s incident reporting process.

Staff who are required to attend any safeguarding meetings **must** give this their priority.

Staff **must** share information as outlined in Section 42 of the Care Act 2014 and West Midlands Adult Safeguarding Policy and Procedures. Please see Appendix 4 for the process.

CWPT and its staff have statutory responsibilities towards prevention of terrorism and radicalisation of individuals. If a staff member becomes concerned that an adult’s behaviour indicates, they may be being drawn into terrorist related activity they must follow the process ‘Action to Take if you Suspect an Individual May be Being Radicalised/Self-radicalised into Extremist Activities’ (Appendix 4).

6. Consultation

In developing this policy consultation and communication occurred with the Safeguarding Operational Group, Safety and Quality Committee, Safeguarding Champions Forum, Local SAP/B’s policy and procedures, subgroups.

7. Implementation

It is the responsibility of all managers to ensure that their staff are made aware of all Trust policies and their working practices reflect them. All staff have a duty to recognise and act on any safeguarding concerns. Training in line with Section 8 will be delivered to staff to help their understanding and implementation of this policy.

Further advice and support can be obtained from the Safeguarding Team.

8. Training and Support

All staff **must complete** the statutory and mandatory safeguarding training as identified in the Trusts Training Needs Analysis specific to their role. This may be delivered via the OnBoarding portal or in a classroom setting.

Prior to commencing employment with the Trust all new starters must complete the safeguarding / PREVENT/WRAP e-learning modules via the OnBoarding portal. This forms part of statutory and mandatory training. They will also receive signposting and awareness training for safeguarding adults as part of their induction.

9. Review

This policy will be reviewed and updated three yearly by the policy author or sooner in line with any changes to service delivery or national guidance

10. Monitoring Compliance

MONITORING COMPLIANCE WITH THE DOCUMENT					
Aspect of compliance or effectiveness being monitored	Monitoring method	Individual / Department responsible for the monitoring	Frequency of monitoring activity	Group / Committee which will receive the findings / monitoring report	Group / Committee / Individual responsible for ensuring that the actions are completed
Safeguarding Concerns / enquiries made	Audits as per safeguarding group audit work plan Data collection	All Trust departments and clinical areas	Ongoing – frequency depending upon nature of service i.e., clinical, corporate	Safeguarding Group	Safeguarding Group
Safeguarding Training	Audit staff compliance of training attendance	Trust Training Department	Quarterly	Safeguarding leads and Safeguarding Group.	Safeguarding Group
Monitoring and review of policy to maintain accuracy and respond to legislative changes and national/local developments	As per Safeguarding Operational Group Work Plan	Safety and Quality	Quarterly or in direct response to specific development	Safeguarding Group	Safeguarding Group

11. References

- Human Rights Act 1998
- Mental Capacity Act 2005: Code of Practice
- NHS Act 2006
- The Care Act 2014
- Social Care Institute for Excellence (2011) Think Child, Think Parent, Think Family: A Guide to Parental Mental Health and Child Welfare
- Health and Social Care Act 2012
- The Home Office, (2018). Criminal Exploitation of Children and Vulnerable Adults: County Lines Guidance.
- The Home Office, (2019). Revised PREVENT Duty Guidance: for England and Wales.
- West Midlands Adult Safeguarding Policy & Procedures: Adult Safeguarding: Multi-agency policy & procedures for the protection of adults with care & support needs in the West Midlands. Version: 2.0 Date of issue: 01st November 2019
<https://www.safeguardingworcestershire.org.uk/wp-content/uploads/2020/06/West-Mids-Policy-and-Procedures-Nov-2109.pdf> Accessed 20/09/2023
- Guidance: Pressure ulcers: applying All Our Health Information for healthcare professionals to protect the public from pressure ulcers {bed sores} (Gov.UK 01st April 2015 updated 09th June 2022
- The Police, Crime, Sentencing and Courts Act 2022
- Domestic Abuse Act (2021) Statutory Guidance (Home Office July 2022 updated April 2023).
- Controlling or Coercive Behaviour: Statutory Guidance Framework (Home Office 05th April 2023).
- Multi-agency statutory guidance for dealing with forced marriage and multi-agency practice guidelines: Handling cases of forced marriage {accessible version} Updated 13 April 2023).
- Coventry City Council. Coventry Safeguarding Adult Board "Safeguarding Adult Interagency Policy and Procedures".
https://www.coventry.gov.uk/downloads/download/4349/safeguarding_adults_-_policy_and_procedures Accessed 20/09/2023
- Warwickshire County Council. Warwickshire Safeguarding Partnership "Safeguarding Adult Interagency Policy and Procedures"
<https://www.safeguardingwarwickshire.co.uk/safeguarding-adults/i-work-with-adults/policy-and-procedures> Accessed 20/09/2023
- Adult Safeguarding: Multi-agency policy & procedures for the protection of adults with care & support needs in the West Midlands. Solihull Council. Solihull Safeguarding Adult Board "Safeguarding Adult Interagency Policy and Procedures"
https://www.solihull.gov.uk/sites/default/files/migrated/StrategiesPlansPolicies_SocialCare_AdultSafeguardingPolicy.pdf Accessed 20/09/2023

12. Trust Associated Documents

This policy and the interagency policies for safeguarding children should be read in conjunction with:

- CWPT, Children's Visiting Policy
- CWPT, Record Keeping Policy (Including Principles and Minimum Standards)
- CWPT, Information Security Policy

- CWPT, Admitting a Young Person to an Adult Ward Policy
- CWPT, Confidentiality and Information Sharing Policy
- CWPT, Disciplinary Policy and Procedure
- CWPT, Domestic Abuse (Clinical) Policy
- CWPT, Domestic Abuse (Staff) Policy
- CWPT, Incident Reporting Policy (Reporting, Managing and Investigating Incidents) (Including Serious Incidents Requiring Investigation (SIRI) Policy
- CWPT, Cultural Directory
- CWPT, Pressure Ulcer Risk Assessment and Management Policy
- CWPT, Managing Personal Relationships at Work
- CWPT, Sexual Safety Within Inpatient Settings Policy
- CWPT, Safeguarding Children and Young People Policy
- CWPT, Policy for the Management of Patient Non-Attendance (Was Not Brought / Failure to Contact) to appointments
- CWPT, Freedom to Speak Up Policy Speak up, listen up, follow up
- CWPT, Allegations Against Staff as Perpetrators of Abuse.
- CWPT, Recording and Completing of Safeguarding Concerns and outcomes of Section 42 Enquiries within Carenotes
- CWPT Standard Operating Procedure – Language & Interpreting Guidance

13. Version Control

Version	Date	Author (Name and designation)	Status (Draft/Approved)	Comments
V2.3	03/07/2015	Chris Evans, Designated Lead Safeguarding Adults and Children	Approved (virtual PRG Chairs Approval)	Amended following review by PRG Chair.
V3.0	August 2018	Mandy Braimbridge Deputy Director of Nursing (Interim)	Approved (virtual PRG Chairs approval)	Amended following review by PRG Chair.
V4.0	February 2020	Annette Dallas Designated Lead Safeguarding Adults & Children	Draft	Updated
V5.0	September 2023	Maxine Nicholls, Head of Safeguarding (children and adults) Julie Vaughan, Lead Nurse for Safeguarding Adults Donna Reeves, Safety & Quality Nurse, Safeguarding Team	Draft	Updated
V5.0	November 2023	Maxine Nicholls, Head of Safeguarding (children and adults) Julie Vaughan, Lead Nurse for Safeguarding Adults Donna Reeves, Safety & Quality Nurse, Safeguarding Team	Approved	Approved by SOG and ratified by PRG.

14. Equality Impact Assessment

Name of new / revised Policy / Strategy / Procedure		
Safeguarding Adults Policy		
Name of Author		
Maxine Nicholls/Julie Vaughan/Donna Reeves		
Directorate		
Corporate		
IDENTIFYING ALL THE AIMS OF THE POLICY / STRATEGY /PROCEDURE		
1.	CONSIDER THE EVIDENCE: Does the Policy / Strategy / Procedure affect one group less or more favourably than another on the basis of the protected characteristics, Please explain?	
	Race	No
	Religion or Belief	No
	Gender reassignment	No
	Sex	No
	Sexual Orientation	No
	Age	No
	Disability (learning disabilities, physical disability, sensory impairment and mental health problems)	The policy is applicable to and appropriate for use with all service users/patients across the Trust regardless of learning disabilities, physical disability, sensory impairment and mental health problems
	Marriage and civil partnership	No
	Pregnancy and maternity	No
2. Where does responsibility for this Policy / Strategy / Procedure lie?		
Chief Executive Officer - Has overall responsibility for the Trust's Safeguarding Adults Policy. Chief Nursing Officer/ Deputy Chief Executive Officer - Has lead responsibility for the implementation of this policy. The Safeguarding Operational Group - Will ensure that policy and practice within CWPT conforms to national guidance and best practice. All General, Senior and Clinical Managers/Clinical staff – Will ensure that this policy is implemented in their areas.		
3. How will the Policy / Strategy / Procedure be put into effect?		
All Trust staff will be made aware of this policy via all user communications, Safeguarding Champions and Directorate Safety & Quality meeting structures.		

4. What criteria will you use to measure the effectiveness of the Policy / Strategy / Procedure?

Via audits in accordance with the Safeguarding Operational Group Audit Plan and monthly training data re staff compliance with Safeguarding Adults training (level dependent upon role).

5. Which other Trust's Policy / Strategy / Procedure does this link with?

- Record Keeping Policy (Including Principles and Minimum Standards)
- Information Security Policy
- Admitting a Young Person to an Adult Ward Policy
- Confidentiality and Information Sharing Policy
- Disciplinary Policy and Procedure
- Domestic Abuse (Clinical) Policy
- Domestic Abuse (Staff) Policy
- Incident Reporting Policy (Reporting, Managing and Investigating Incidents) (Including Serious Incidents Requiring Investigation (SIRI) Policy
- Pressure Ulcer Risk Assessment and Management Policy
- Sexual Safety Within Inpatient Settings Policy
- Managing Personal Relationships At Work
- Safeguarding Children and Young People
- Policy for the Management of Patient Non-Attendance (did not Attend / Was Not Brought / Failure to Contact)
- Freedom to Speak Up Policy Speak up, listen up, follow up
- Allegations Against Staff as Perpetrators of Abuse.
- Recording and completing of Safeguarding Concerns and outcomes of Section 42 Enquiries within CareNotes
- Standard Operating Procedure – Language & Interpreting Guidance
- Standard Operating Procedure – Recording and completing of Safeguarding Concerns and outcomes of Section 42 Enquiries within CareNotes.

6. How has there been involvement and/or consideration of stakeholders from different groups in the development of this Policy / Strategy / Procedure YES/NO.

This policy has been developed in Consultation with the Safeguarding Operational Group, Safeguarding Team and Safeguarding Champions Forum.

7. How will information pre and post consultation be available e.g., on the Intranet?

On the Trust Intranet via the Policies and Toolkits pages and Directorate Safety & Quality meeting structures.

IMPACT CHECKLIST

Are there factors other than elements of diversity which might help explain any adverse impact or use of this Policy / Strategy / Procedure e.g., the need to have a driving license for a driving post (recruitment policy)

N/A

How will the proposed Policy / Strategy / Procedure increase equality of opportunity for some groups or individuals? If so, explain how? N/A
Are you confident that the Policy / Strategy / Procedure is lawful and can be justified? Explain This policy has been developed in line with latest national guidance and legislation.
ALTERNATIVES CHECKLIST
Are there any alterations or additional measures which would remove or reduce adverse impact without effecting the Policy / Strategy / Procedure overall aims? N/A
If this Policy / Strategy / Procedure positively impacts a particular group, will any benefit to them outweigh the costs other groups? please explain. N/A
If you do not make above alterations, what are the consequences for the Partnership Trust or those individuals/groups? e.g., employment tribunal N/A
DECIDE WHETHER TO ADOPT THE POLICY / STRATEGY / PROCEDURE
What are the main findings of the consultation? This policy is required to ensure that when adult abuse is disclosed, suspected or apparent, Trust staff members are able to respond effectively and appropriately.
How will you change the policy / strategy / procedure to consider feedback from consultation and other information gathered? Further changes to the policy would be made as a result of significant change to local national guidance and/or legislation.
Who will be involved in the decision-making process of whether to adopt the Policy / Strategy / Procedure and why? Safeguarding Operational Group, because all directorates are represented, and the Chair is the Chief Nursing Officer/ Deputy Chief Executive Officer and has lead responsibility for the implementation of this policy.
MONITORING
Consider piloting the Policy / Strategy / Procedure to assess impact why Yes or No This is a reviewed policy, not a new one therefore no pilot required.

What criteria will be used to assess the effects on equality of opportunity and who will carry out the assessment?

Sign off by our Head of Equality, Diversity & Inclusion.

How often will the Policy / Strategy / Procedure be reviewed and by whom?

Three yearly or whenever there is significant change to service delivery and/or national guidance, by policy author(s).

Date of Assessment: (dd/mm/yyyy)	07/09/2023
Names and designations of the 3 people who contributed to this full EIA assessment	1. Donna Reeves – Safety & Quality Nurse – Safeguarding Team 2. Maxine Nicholls – Head of Safeguarding 3. Julie Vaughan – Lead Nurse for Adult Safeguarding
Head of Equality, Diversity & Inclusion	Rano Bains
Date of Ratification:	01/11/2023
Date of next review:	01/11/2026

If you have identified a potential discriminatory impact on this procedural document, please refer it to the author of the policy or strategy, together with any suggestions as to the action required to avoid / reduce this impact. For advice in respect of answering the above questions, please refer to the guidance notes. **If the document affects one group less or more favourably, you MUST complete the full EIA form (i.e., if you have answered 'Yes' to any of the above).** The full EIA form can be obtained from the Equality, Diversity & Inclusion Department/website.

This policy, strategy, procedure, or function has to go to the Head of Equality, Diversity & Inclusion for final sign off.

Please return a copy to rano.bains@covwarkpt.nhs.uk

Appendix 1

1. Allegations Against Staff Members

If the alleged perpetrator is a staff member or employed on a bank or agency, then the Safeguarding Procedure will be implemented as normal. i.e., if an alleged criminal act has occurred, the criminal investigation will take precedence and may be in conjunction with disciplinary procedures. NB: where employed staff have been subject of safeguarding and are perpetrators of abuse, they should be referred to the Disclosure and Barring Service (DBS) for consideration of barring from working with vulnerable adults and children in the future. A referral may also need to be made to the relevant professional bodies.

The process below should be followed and read in conjunction with CWPT policy Allegations against Staff, in a Position of Trust, as Perpetrators of Abuse (September 2023)

2. Personal Relationships

There may be instances where staff members have personal relationships, present or historical, with service users / ex-service users:

- Where staff members are in a current personal relationship with a service user, they must inform their manager immediately who will ensure appropriate arrangements can be put in place.
- Where staff members have been in a historical personal relationship with a service user, they must inform their manager immediately who will ensure appropriate arrangements can be put in place.
- Where staff members have a personal relationship with a service user where they have previously had a therapeutic relationship with a service user, they must inform their manager immediately who will ensure appropriate arrangements can be put in place.

3. Staff having relationships with previous patients of the Trust

- It is unacceptable for any member of staff to use patient information to contact the said named patient for personal reasons / gain. This is in breach of information governance and will lead to a disciplinary that may lead to dismissal from the service.
- Whilst the Trust accepts that there are occasions when staff form relationships with people who have previously received a service from the Trust. The Trust needs to establish and assess the basis for these relationships in order to be fair and equal and not to allow the staff member to misuse the power of their present or previous role or the patient to manipulate their contact with staff members.
- It is this power differential that results in patient vulnerability. The Nursing & Midwifery Council (NMC) believes that sexual/personal relationships at any time with people in the care of a nurse or midwife, past or present, are always potentially abusive. Therefore, the Trust have decided that Trust staff, must not engage in any sexual/personal activity with any person in their care or make any sexual advances verbally, physically or by innuendo. In the event of a complaint, it will be for the member of staff to show that he or she has not abused or exploited any professional relationship.

- It is not uncommon for a member of staff to have sexual/personal feelings towards a person in their care but acting on such feelings is always unprofessional and potentially damaging to the person the member of staff caring for.

To meet this requirement, any Trust staff member who has formed a relationship with a present or previous service user needs to Inform their manager as soon as they have become aware that their partner is or has been a patient of the service. This must be followed up with an in depth conversation to establish that the imbalance of power between the past, current and present service user is not at a level that could be considered abusive.

This conversation should cover:

- When the professional relationship ended and how long this relationship lasted.
- Whether the professional relationship consisted of a significant power imbalance
- Whether the former person in their care, was or still is particularly vulnerable
- Whether they are exploiting a power imbalance or the knowledge or influence that they gained through the professional relationship to develop or progress the relationship
- Whether they are treating or are likely to treat any other members of the person's family.

In the event of a complaint, it will be for the employee to show that he or she has not abused or exploited any professional relationship. They will also need to show that all the issues above have been considered and that they have sought appropriate advice. If it is alleged that a staff member is having, or has had a non-therapeutic relationship with a service user or ex-service user as defined above without informing their manager the staff member will be subject to Person in a Position of Trust PiPoT procedures, including the activation of the Trusts ' Allegations against Staff, in a Position of Trust, as Perpetrators of Abuse policy

4. Police Involvement

The service user must be asked if they want to involve the police where a possible criminal offence has occurred. It is important to ensure that this happens immediately, and the criminal investigation takes precedence over a safeguarding investigation.

In situations where the service user is unwilling to make a formal complaint to the police, the Safeguarding Manager should give careful consideration as to whether they have a duty to report the matter to the police directly. The decision should be based on the risk to the service user, the risks to others and the seriousness of the allegation. If the service user lacks capacity to consent, a decision should be made on a best interest's basis (see section 3.5 Mental Capacity Act for more information). All decisions and actions should be fully recorded.

5. Liaison with Regulatory Agencies

Whenever an investigation is being considered, contact should be made with any regulatory or funding agencies involved, for example the Care Quality Commission, Health and Safety Executive.

6. Liaison with Commissioners of Services

Services may be commissioned by Social Care, GP Consortia, Supporting People, or other partner agencies. Services may be commissioned wholly by one agency, separately by more than one agency or jointly. All commissioners, including people who fund their own care and personal budget holders, should be advised of investigations, and invited to participate in safeguarding processes as appropriate.

7. Responsibility of Local Authority Contract & Commissioning Team/GP Consortia

Contract and quality monitoring by the Contracts and Commissioning Team includes the review of issues relating to safeguarding of vulnerable adults. It is imperative that any concerns raised during an investigation regarding any contracted service are notified to the relevant Local Authority/Primary Care Trust Contracts and Commissioning Teams.

8. Supporting Staff

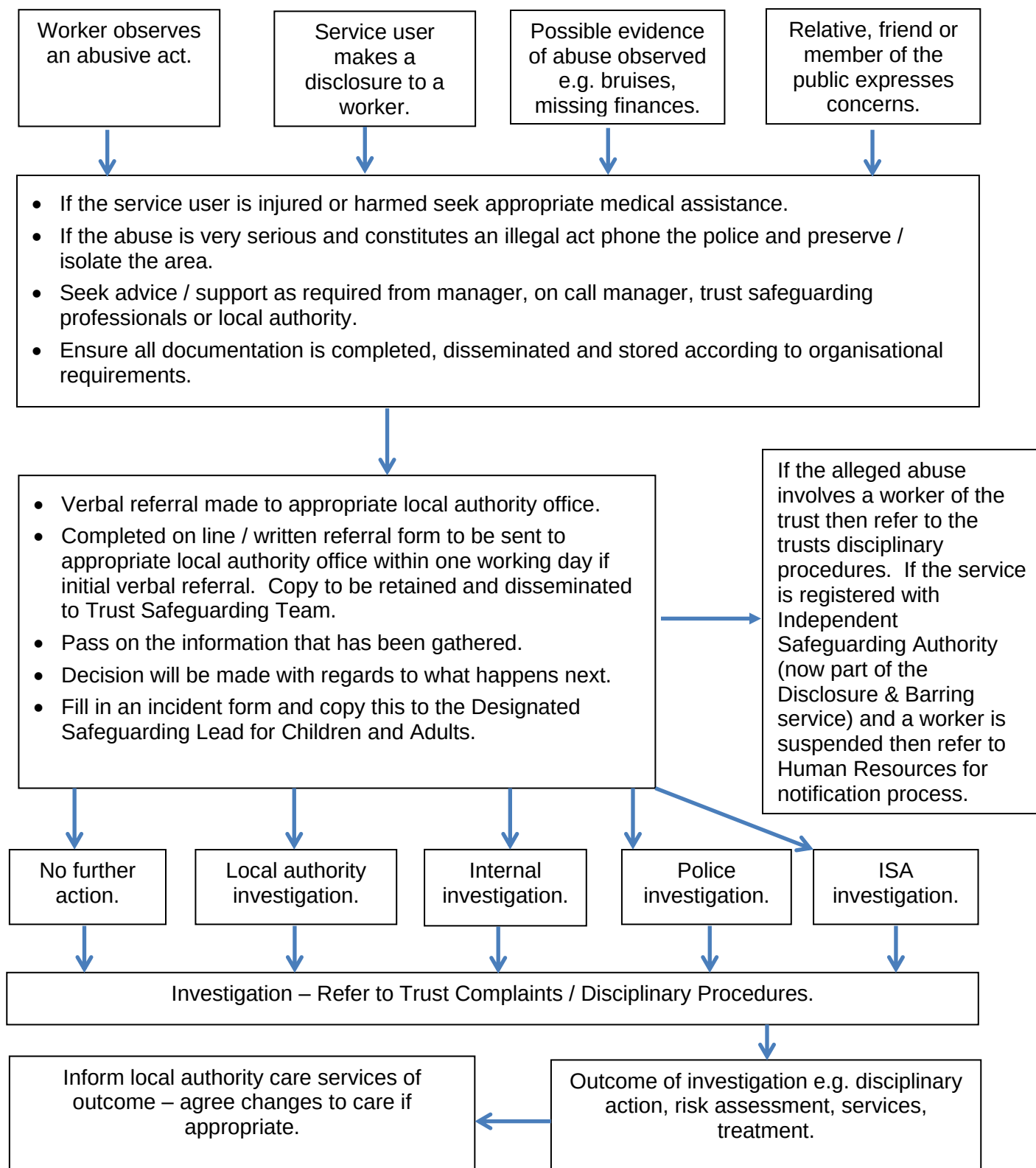
Involvement in Adult Safeguarding work may be stressful for staff members that need to empathise with victims and carers, confront abuse issues, resolve conflict, and establish support and protection. It is important that the impact on staff is recognised, and they have appropriate opportunities for support through management or professional supervision. Where necessary staff should be offered access to confidential counselling.

9. Freedom to Speak Up (Colloquially known as “Raising Concerns / Whistle Blowing” Worker Disclosures.

Our policy provides a framework for speaking up, listening up and following up. Workers who have concerns about abuse in their workplace should notify their line managers or appropriate senior lead in the first instance. If a manager is implicated or they feel unable to report to their line manager, then there are other routes staff can use outlined in the policy including the Freedom to Speak Up Guardian and Speak Up Champions. Staff who report allegations or suspicions of abuse, should receive thanks, acknowledgement, and support in line with this policy.

Appendix 2

Process for Reporting a Safeguarding Concern (Non-Mental Health Services)



Safeguarding Form Guidance

(For use in Secondary Care Mental Health Services ONLY)

Name of form	When to use it
Safeguarding concern	An abusive act is witnessed. Disclosure from a third party. Adult makes a disclosure. Evidence of possible abuse or neglect. Suspicion or concern that something is not right.
Chronology template	Record ALL safeguarding activity e.g. telephone calls, discussions with clients, reasons made to close concern.... (make note in body of Carenotes: SAFEGUARDING-see Chronology Template).
Sec 42 Enquiry Report	To evaluate the outcome of the enquiry and decide what action is needed and demonstrate how the adults' views, needs and wishes have been considered.
Safeguarding meeting minutes template	Ensure that an accurate and comprehensive record of the safeguarding meeting is made.
Safeguarding plan	A Safeguarding Plan must be used when the risk of abuse or neglect is ongoing, complex and/or unstable.

Safeguarding Adult Chronology

Purpose: To show a timeline of events/recording in a table format.

Name of adult:

Date of birth:

NHS No:

Date dd/mm/yy Time	Description of Safeguarding activity	Contact with whom i.e. name, and role of person	Actions identified (including who is responsible for completing action)	Name & Designation (staff recording entry)
EXAMPLE	EXAMPLE	EXAMPLE	EXAMPLE	EXAMPLE
23/10/2019 14.25	Concern raised and discussed with client. Safeguarding form completed (UDF). Manager Joe Brown informed via email.	the client-Paul Jones	Joe Brown to complete Decision section on safeguarding form	Albert Smith OT

[illegible]

S42 Enquiry Report

Name of Professional undertaking S42 enquiry	
Contact details of Professional	
Name of Decision Making Manager	
Contact details of Decision Making Manager	
Date report completed	
Date report submitted to Decision Making Manager	

Use the information recorded within the chronology template to inform this S42 Enquiry Report

The Purpose

- To detail the findings of the enquiry
- To establish facts
- To evaluate the outcome of the enquiry and decide what action is needed
- To demonstrate how the Adults needs, views, beliefs, feelings and outcomes have been considered
- To review and assess risk and need for protection
- To establish whether there is a need for a safeguarding plan
- To establish whether there is a need for a safeguarding meeting
- To determine whether harm took place and what if any actions are being taken.

Ensure each abuse type is stated along with the description of harm
Abuse 1: Abuse 2: Abuse 3: (Add more if required)
Views, wishes and feelings of the adult (via advocate/representative where necessary)
Findings from each agency/person
Evaluation of findings Refer to the chronology if more detail is needed. Highlight any significant gaps in information, any anomalies or differences of opinion. Ensure that you clarify what is fact and what is opinion in this report. Any written correspondence relating to the safeguarding enquiry must be uploaded to the CWPT Safeguarding tab within Carenotes). State which evidence supports the allegation/s and which refutes them or if there is insufficient evidence to reach a conclusion. Show how the evaluation of evidence informs the recommendations detailed below.

Recommended outcome of safeguarding enquiry

For each abuse type was harm/abuse/neglect identified and was any action taken, or is being planned to be taken, to address that harm/potential harm?

Type of abuse	Was abuse identified?	Recommended Action/ActionTaken
Abuse 1:		
Abuse 2:		
Abuse 3:		

Other recommendations (i.e. those not related directly to the enquiry)

Type of abuse	Risk of harm remained, reduced or removed	How have you reached this conclusion?	Is ongoing safeguarding plan required?	If YES who will manage the plan?
Abuse 1:				
Abuse 2:				
Abuse 3:				

Decision Making Manager's comments

--

Decision Making Manager's Approval

Record of approval of the S42 Enquiry Report by the Decision Making Manager.

Decision Making Managers Name	
Job Role/Team	

Approval Date	
---------------	--

Safeguarding Meeting Minutes

Confidentiality and signing-in sheet for Safeguarding Adults Meetings

Ref:

Team address

Tel:

Web:

Date

Safeguarding Meeting re:

DOB:

Confidentiality Reminder

The documents, other materials and discussion at this meeting are highly confidential and can only be used in ways agreed at the meeting in order to protect and safeguard an adult with care and support needs. The notes of the meeting are confidential in the same way and should be stored safely and securely and not distributed without reference to the Chair of the meeting.

You have been asked to sign the attendance list below which establishes your agreement to comply with the Confidentiality Reminder above.

Name	Signature	Contact details

Ref:

Team address

Tel:

Web:

Safeguarding Adults (Section 42 Enquiry) – Meeting Minutes

STRICTLY CONFIDENTIAL

These minutes are strictly confidential and must not be photocopied. Permission must be obtained from the Chair of the meeting before they are shown to other people.

PLEASE NOTE: Requests for amendments to these Minutes should be forwarded in writing to the Chair of the meeting, within seven days of the circulation date; otherwise they will be taken as an accurate record.

Name (of alleged victim)	<i>Caution: Some agencies may need the individuals address or DOB to identify the person</i>
Address	
DOB	
NHS number	
Date of meeting	
Date Minutes circulated	
Venue of meeting	
Present	List everyone who attends the meeting, note if they are only present for part of the meeting. Detail the organisation they are from, their role
Apologies	Have they sent someone in their absence or sent a report?
Non Attendees	List everyone who was invited, did not come and did not give apologies as above
Copies of Minutes to	Additional to those present or who have sent apologies
Introductions & housekeeping	The Chair advised that this Safeguarding Adults Meeting was being convened under the relevant multi-agency safeguarding procedures and reminded everyone that the proceedings were confidential. If any information needs to be shared, this should first be checked with the Chair. Minutes of the meeting will be circulated to all attendees and those who have given apologies.

Summary of situation	Detail suspected type of abuse, suspected perpetrator, suspected duration of abuse, place of abuse etc. Summary from care coordinator
Mental capacity (decision specific)	When was capacity assessed? Ensure capacity has been considered re specific safeguarding concerns/issue.
Substantial difficulty	If the person is deemed to have capacity do they have substantial difficulty in understanding the safeguarding process
Advocate	Has the person got an advocate?
View of the adult with care and support needs	MUST be completed
Information from each agency and reports received	Consider all professional information at the meeting and any reports received
Views of next of kin and/ or advocate (IMCA) or person with care and support needs	
Risk assessment/ appraisal	Is the CPA up to date How does the meeting appraise the risk, i.e. accepted, reduced or removed
Next steps/agreed actions	

Agreed actions			
	Action to be taken	Responsible person Have they agreed to this action (Y or N)	Date for completion or review <i>Please specify</i>
1	Who is going to inform the client of the outcomes?		
2			
3			
4			
5			
6			
7			
8			
9			
10			

AOB	
Date/time and venue of next meeting	
Signature	
Name and designation	
Date	

Safeguarding Plan

Forename		Surname		Date of Birth	
Gender		Ethnic Origin		Religion / Faith	
Permanent address					

A long term or ongoing safeguarding plan is used when there is a need for a greater level of scrutiny and review, usually within a multi-agency context. This could include:

- the risk of abuse or neglect is ongoing, complex, unstable
- coercion, undue influence, or duress add to the complexity and uncertainty of the risk,
- the risk cannot be managed appropriately or adequately by other processes.

Purpose of plan

- To plan actions to prevent further harm/abuse or neglect.
- To plan actions to promote the safety and wellbeing of the adult,
- To plan actions to promote resolution & recovery from the experience of abuse or neglect
- To ensure all the above take into account the protective measures, the Adults wishes, beliefs, values and outcomes
- **Positive risk taking should be considered.**

Part 1 - Individuals involved in this plan

Name of worker co-ordinating/reviewing the plan	
Name of manager	
Start date of plan	

Who needs to be involved or kept aware of this safeguarding plan?

Name	Role	Contact Details

Part 2 - Risk management, safety, resolution and recovery

List protective factors and the difference they make to reduce the risk(s). List the adults strengths, ability to protect themselves, promote their own personal safety and any networks they can access to achieve safety from the harm.

Presenting risks and actions to be taken to reduce the risk

Presenting Risk	Protective Factors	Action to be taken to reduce the risk	By whom	By when
e.g. (client) Paul Jones's brother George is harassing Paul for money. George has a history of substance misuse and has recently been released from prison.	Julie (Pauls sister) is aware that George is harassing Paul with whom she has a good relationship.	Liaise with safeguarding lead at bank to make them aware of potential coercion by George.	Jas Dhillon (Care Co-ordinator)	27/11/2019
		e.g. client wants to retain relationship with brother and maintain control of his own finances		

How does the above plan promote the Adult's wishes, beliefs, feeling, views and desired outcomes?	
---	--

Goals - Resolution and recovery

What does the Adult want to do to achieve resolution and/or recovery	Action to promote resolution/recovery	By whom	By when
E.g. Improve his confidence in managing his money.	Paul to attend session arranged by bank to help customers to understand money management.	Paul to contact bank to enquire about course, supported by sister Julie.	01/01/2020
How does the above plan reflect the Adult's wishes, feelings, view and outcomes about how they want to recover from the harm or neglect and/or how they want things resolved?			
e.g. Paul wants to feel in control of his own finances so that he does not feel pressured into giving his money to his brother.			

Other information relevant to the safeguarding plan

Review Date 1	Review Date 2	Review Date 3	End Date of Plan
/	/	/	/

Recording and completing of Safeguarding Concerns and Outcomes of Section 42 Enquiries within Carenotes

Accessing the form.....	Page 1
<u>Mandatory Fields</u>	Page 2
Raising a concern.....	Page 3
Management Decision.....	Page 4
Completing S42 Enquiry.....	Page 5

Staff affected: Mental Health Professionals
Date created: 25th June 2020
Date updated: 31st July 2023
Locality: Coventry and Warwickshire only
Client Group: Mental Health Services
Client Age Group: 18 and over

Accessing the Concern and Enquiry Form

The form is available under the Safeguarding tab

The screenshot shows the Carenotes interface for a patient named Donald Duck. The patient's details include: Born 02/12/1976 (41y), Gender Male, NHS No 609 432 1561, and Address: Water Hall, Great Canfield, DUNMOW, Essex, CM6 1JR. The 'CWPT Safeguarding' tab is highlighted with a red circle. Below the tabs, there are radio buttons for 'Active documents', 'Latest spell', and 'Entire record'. A 'Create a new' dropdown menu is visible, and a green arrow points to it with the text: 'To add a new Concern, select "Safeguarding Concern and Enquiry" here'.

Fig 1

Raising a Concern: Select the "Safeguarding Concern and Enquiry v4" form as in above picture; by clicking the dropdown box by "Create a new". The form will appear in a few seconds.

Please remember it is your responsibility as a practitioner to raise a concern

Decision making managers actioning a raised Concern or completing the Enquiry: You can access an already raised Concern by selecting the one in the list as in Figure 2. The form will appear in a few seconds.

The screenshot shows the 'carenotes' interface for a patient named DUCK, Donald (Mr). The patient's details include: Born 02/12/1976 (41y), Gender Male, NHS No. 609 432 1561, and Address: Water Hall, Great Canfield, DUNMOW, Essex, CM6 1JR. The form has tabs for Back to Search Results, Notes, and Summary. Below the patient details is a table with fields for GP, Consultant, Primary Worker, Mobile number, Local ID, Gender, Legal Status, and Primary contact. A 'Case Notes' section is also visible. A purple cloud annotation points to the 'Summary' tab, stating: 'Raised Concerns will be listed here for Manager/Care Coordinator Action'.

Fig 2

MANDATORY FIELDS

Rules attached to the Concern and Enquiry form: The Nursing, Safety and Quality team will utilise the information to ensure requirements are met for local and National data requirements. This means certain fields on the form are **mandatory**. The form has colour coded boxes to advise which fields need completing, at what stage and by whom.

The screenshot shows the 'carenotes' interface for a patient named DUCK, Donald (Mr). The patient's details include: Born 02/12/1976 (41y), Gender Male, NHS No. 609 432 1561, and Address: Water Hall, Great Canfield, DUNMOW, Essex, CM6 1JR. The form has tabs for Save and Cancel. Below the patient details is a table with fields for GP, Consultant, Primary Worker, Mobile number, Local ID, Gender, Legal Status, and Primary contact. A 'Safeguarding Concern and Enquiry' section is also visible. A legend at the bottom explains the color coding: Blue Highlighted (Mandatory at point of raising a concern. The form cannot be submitted to your manager until the fields have been completed), Green Highlighted (Mandatory for managers to complete in deciding to proceed to Section 42 Safeguarding Enquiry. If the concern does not go on to enquiry, it can be submitted once the green (and blue) fields are complete), and Pink Highlighted (Mandatory for Manager to complete questions only when Concern leads to Enquiry. These fields need completion before the form can be submitted).

Fig 3

BLUE: Mandatory when submitting the form. The manager can update this information at a later stage.
GREEN: Decision making Managers need to complete these fields when determining whether to proceed to Enquiry.
PINK: Decision making Managers need to complete these fields when completing S42 Enquiry.

Raising a Concern

Staff raising a concern: The top half of the form is to be completed by staff raising a safeguarding Concern; up to the section of the decision to proceed to Section 42 Safeguarding Enquiry. The more information that is provided will help build a better picture of the circumstances for the decision making manager to decide whether to proceed to Section 42 Safeguarding Enquiry and the patient to receive the right level of care. All boxes highlighted blue must be completed at point of raising a concern.

Entering Dates: You need to click the field header. A Calendar will appear from which you can select the date.



A. Safeguarding Concern

Abuse Allegation/Concern Date: *

Fig 4

Abuse type and location of abuse: You are able to document 2 abuse types. With each abuse type, there is a requirement to detail the location(s) of each abuse type independently. See figure 5. If there isn't a second abuse type – an option of “2nd abuse type not identified” needs to be entered.

Type of Abuse	
	Abuse Type 1
	- Please Select -
Location 1 of abuse type 1 *	- Please Select -
Location 2 of abuse type 1	- Please Select -
Location 3 of abuse type 1	- Please Select -
if other location specify:	
	Abuse Type 2
	- Please Select -
Location 1 of abuse type 2 *	- Please Select -
Location 2 of abuse type 2	- Please Select -
Location 3 of abuse type 2	- Please Select -
if other please specify:	

Fig 5

Once the details have been entered, press “Save” located at the top of the form. You must ensure you email/ contact the decision making Manager to inform that a Safeguarding Concern has been raised. Please be aware if they are out of office, it is **your responsibility** to advise the Deputy so that action can be immediate for the safety of the patient.

If you have part completed the details, you are able to save your changes, and come back to at a later point. Hit Save, then you can always find it again as in Figure 2

The decision to proceed to Section 42 Safeguarding Enquiry

Decision making manager: This decision is for completion of the decision making Manager. This could be the manager of the nurse or social worker or Allied Health Professional that is caring for the patient.

1. Access the raised Concern by selecting it; as in Figure 2.
2. Once the form appears it is in a read only state.
3. To begin updating, click Edit at the top of the form as in Figure 6 below.

Fig 6

The green fields are **mandatory** at this stage.

Decision to proceed to Section 42 Safeguarding Enquiry: Select “Enquiry” within ‘Management Decision’. Complete all green boxes. You are able to add additional information to the Concern form if required. Proceed to the Section 42 Safeguarding Enquiry section of the form.

Entering Dates: You need to click the field header. A Calendar will appear from which you can select the date.

Fig 7

If you have part completed the details of the Section 42 Enquiry, you are able to save your changes, and come back to at a later point. Hit Save, then you can always find it again as in Figure 2

Decision not to proceed to Section 42 Safeguarding Enquiry: Document this as “NFA” within field ‘Management Decision’ and complete the ‘NFA Reason’. Once, you have entered your details (Name, Post and Date); the form is complete. Press ‘Save’ at the top to save your changes and exit the form. The remainder of the form does not need attention or completion. Any information in the Section 42 Enquiry section will not be considered in data returns for cases not proceeding to Enquiry.

Timely Manager Action: Managers when out of office are required to ensure their “out of office” is completed appropriately so staff raising a concern can be informed to notify the Deputy of raised concerns.

Outcome of Section 42 Safeguarding Enquiry

This section as in Figure 8 needs completing **ONLY** if the case went on to Section 42 Safeguarding Enquiry. This section is to be completed by the decision making Manager only.

The screenshot shows a web-based form for 'Management Decision to Proceed to Section 42 Safeguarding Enquiry'. At the top, there are 'Save' and 'Cancel' buttons. The form is for a patient named 'DUCK, Donald (Mr)' with a date of birth of '02/12/1976 (41y)' and gender 'Male'. The address is 'A02, Water Hall, Great Canfield, DUNMOW, Essex, CM6 1JR'. The form includes a section for 'PLEASE INFORM YOUR MANAGER THAT THERE IS AN OPEN SAFEGUARDING CONCERN REQUIRING MANAGER ACTION AND CC. THE EMAIL TO THE SAFEGUARDING INBOX ON Safe.Guarding@covwarkpt.nhs.uk MANAGERS ONLY TO COMPLETE THE REMAINDER OF THE FORM'. Below this, there are fields for 'Management Decision' (with a dropdown menu), 'NFA Reason' (with a dropdown menu), and 'Decision Making Manager Details' (with fields for Name, Role, and Team Name). A red arrow points to the text 'MANAGER TO COMPLETE THIS SECTION ONLY IF THIS CONCERN LEADS TO AN ENQUIRY.' which is located above the 'OUTCOME OF SECTION 42 ENQUIRY' section. This section includes fields for 'Any changes made to Section A (Concern) details', 'Changes made to Section A', 'Dates of Safeguarding Meetings held' (with fields for Date 1, Date 2, and Date 3), 'Brief description of the findings of the Section 42 Enquiry', and 'Wishes' (with fields for Wish 1, Wish 2, and Wish 3, each with a dropdown menu for 'Please Select -').

Fig 8

Decision making Managers are able to save the form and come back to at a later stage, by clicking Save and returning to the form as in Figure 2.

Mandatory Fields at Enquiry stage: Decision making manager are to ensure the boxes coloured pink are completed prior to the completion of the Enquiry section of the form.

Clients Wishes: See Figure 9, Wishes are required to be listed here. You are able to document up to 3 wishes and provide the outcome of each wish. As multiple wishes can be listed, the consolidated outcome needs to be entered to the field where the arrow is pointing.

MANAGER TO COMPLETE THIS SECTION ONLY IF THIS CONCERN LEADS TO AN ENQUIRY.

OUTCOME of SECTION 42 ENQUIRY

Any changes made to Section A (Concern) details

Changes made to Section A

Dates of Safeguarding Meetings held:

Date 1 Date 2 Date 3

Brief description of the findings of the Section 42 Enquiry

Up to 3 wishes can be listed

Wish 1 Wish 2 Wish 3

Wish Established:

State wish if other

Has the adult (alleged victim) wishes been met

Outcome of the adult (alleged victim) at risks wishes

Overall, has the Adult (alleged victim) wishes/outcomes been met?

If not, why not?

MANDATORY CONSOLIDATED RESPONSE

Fig 9

Risk Assessment Outcome: Decision making Managers will need to ensure they reference Abuse Type 1 and Abuse Type 2 respectively when they complete the Risk Assessment Outcome. Please see Figure 10. Abuse type 1 and Abuse type 2 are documented above in the Concern section of the form.

Outcome of the adult (alleged victim) at risks wishes

Overall, has the Adult (alleged victim) wishes/outcomes been met?

If not, why not?

Has there been a IMHA/ IMCA/ Section 68 Advocate/Representative involved?

If Yes, Advocate details

In relation to abuse type 1, the risk assessment outcome was

In relation to abuse type 2, the risk assessment outcome was

Managers need to ensure this relates to Abuse Type 1

Managers need to ensure this relates to Abuse Type 2

Fig 10

Outcome of Section 42 Safeguarding Enquiry: Decision making managers are required to state if the risk to the client (alleged victim) has been reduced/ remains or removed. Each outcome is attached to each abuse type as listed in the Concern. Ensure to respond to each abuse type respectively. Ensure complete remaining fields as appropriate.

If Yes, Advocate details

Regarding abuse type 1 documented above. Following the subsequent services/actions, the risk of abuse to the Adult (alleged victim) is

Does the Adult (Alleged Victim) feel safer as a result of the interventions/services offered?

If not, why not?

Has the Adult (Alleged Victim) died or been seriously harmed where abuse/neglect is suspected to be a factor?

Has the Adult (Alleged Victim) been informed of the outcome?

If not, why not?

Regarding Abuse type 2 documented above. Following the subsequent services/actions, the risk of abuse to the Adult (alleged victim) is

Managers need to ensure this relates to Abuse Type 1

Managers need to ensure this relates to Abuse Type 2

Fig 10

Advocacy: If the concern went to Enquiry and the client did not have capacity or have substantial difficulty in making their views known, staff should ensure an appropriate person represents the adult (alleged victim). This could be a family member or advocate. See figure 11.

Outcome of the adult (alleged victim) at risks wishes	
Overall, has the Adult (alleged victim) wishes/outcomes been met?	
If not, why not?	
Has there been a IMHA/ IMCA/ Section 68 Advocate/Representative involved?	
If Yes, Advocate details	

Fig 11

Informing the Adult (alleged victim) of the outcome:

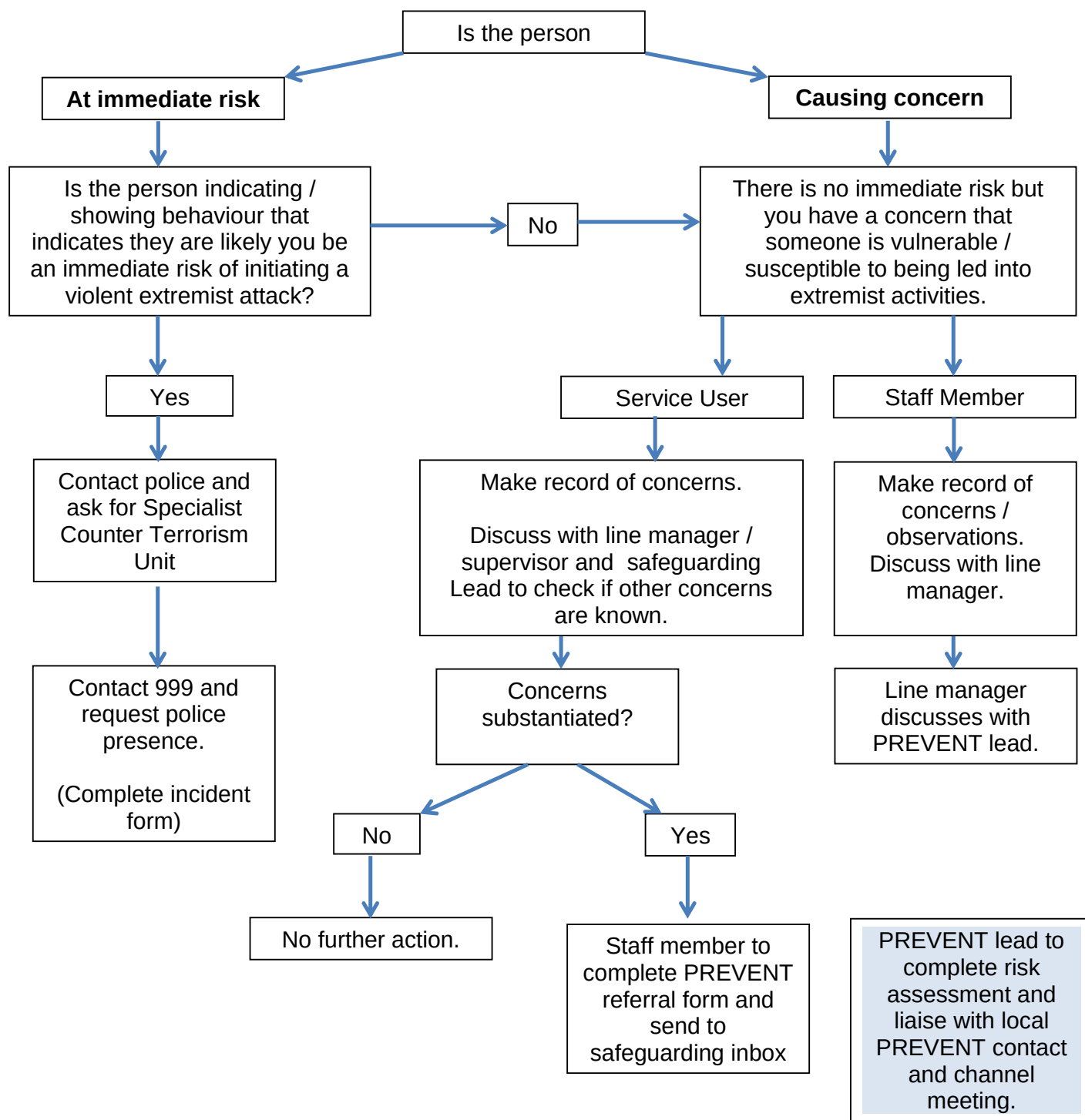
Staff need to include the alleged victim (or advocate if there is one in place) in the process of safeguarding and ensure they are informed of the outcome, communicated so that it is understood by the adult (alleged victim). See Figure 12.

Outcome of the adult (alleged victim) at risks wishes	
Overall, has the Adult (alleged victim) wishes/outcomes been met?	
If not, why not?	
Has there been a IMHA/ IMCA/ Section 68 Advocate/Representative involved?	
If Yes, Advocate details	
In relation to abuse type 1, the risk assessment outcome was	In relation to abuse type 2, the risk assessment outcome was
Regarding abuse type 1 documented above. Following the subsequent services/actions, the risk of abuse to the Adult (alleged victim) is	Regarding Abuse type 2 documented above. Following the subsequent services/actions, the risk of abuse to the Adult (alleged victims) is:
Does the Adult (Alleged Victim) feel safer as a result of the interventions/services offered?	
If not, why not?	
Has the Adult (Alleged Victim) died or been seriously harmed where abuse/neglect is suspected to be a factor?	
Has the Adult (Alleged Victim) been informed of the outcome?	
If not, why not?	

Fig 12

Completion of Section 42 Safeguarding Enquiry: The Closure of the Safeguarding Enquiry will be noted once the 'Date of Management Decision to Close Section 42 Enquiry' box has been completed with the appropriate date. The decision making Manager will need to complete the Name and Post fields.

Action to Take if You Suspect an Individual Is Being Radicalised /Self-radicalised into Extremist Activities



Additional Information for Staff:

- Staff should be familiar with the Trust and the Local SAP /B"s with whom the Trust works collaboratively with.
- Staff need to consider who else is in the household and if a Multi Agency Safeguarding Hub (MASH) contact / referral is necessary, or if a safeguarding concern is required for other adults with care and support needs.
- Staff must follow the above procedures when there are adult safeguarding concerns.
- Confidentiality laws are maintained within health; however confidentiality in adult safeguarding that are governed by The Care Act 2014.
- All staff must attend the mandatory and statutory adult safeguarding training that is identified within their role and appropriate training for PREVENT. Please See the Trusts training Needs analysis.
- It is each individual staff member's responsibility to ensure they access the appropriate supervision relating to adult safeguarding.
To produce and provide written report for safeguarding meetings using agreed pro forma within their local policy and procedures.